



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : جمالات عاشور مصطفى حسن

المحافظة : الجيزة الحي / المدينة : الهرم

العنوان : ش الاربعين - كعبيش - فيصل

رقم البطاقة : 25708140102862 رقم التليفون : 01111722481

العمر (تاريخ الميلاد) : 14-08-1957 المهنة : ربه منزل

رقم التليفون : 01111722481

تاريخ تسجيل الإستمارة : 28-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....) المقر بما فيه التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: جمالات عاشور مصطفى حسن National ID: 25708140102862	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient's Name: Jamalat Ashour Mostafa Hassan
Date: 2026-06-03
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MRI EXAMINATION OF LUMBOSACRAL SPINE

Technique :

- Sagittal T1WI, T2WI
- Axial T2WI

Findings:

- Straightened lumbar curve
- 1st degree anterolisthesis of L4 over L5
- Spondylodegenerative changes manifested by:
 - Antero-marginal osteophytic lipping of the opposing lumbar vertebral endplates.
 - Reduced height & normal bright T2 signal of lumbar discs.
- L3-4, L4-5 and L5-S1 diffuse posterior disc bulge is seen effacing the ventral epidural fat, flattening the thecal sac and partially encroaching upon inferior recess of the corresponding neural exit foramina.
- L2-3 posterior disc protrusion is seen effacing the ventral epidural fat, flattening the thecal sac and partially encroaching upon inferior recess of the corresponding neural exit foramina.
- L4-5 facetar arthropathy.
- No vertebral structural collapse or marrow signal alteration.
- Normal MR signal of the conus medullaris & cauda equine nerve roots.
- No paravertebral masses or collections

OPINION:

- 1st degree degenerative anterolisthesis of L4 over L5
- Lumbar spondylotic changes with multilevel disc lesions as described.

Dr.Assem Ahmed

Dr.Assem Ahmed

جمهورية مصر العربية
بطاقة تعريف شخصية

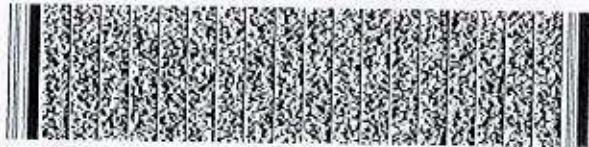


جماليات
عاشور مصطفى حسن
٢٣ بلوك ١٥١٦ امام المرتفعات الخضراء
٦ اكتوبر اول - الجيزة

١٩٥٧/٠٨/١١

٢ ٥٧ ٠٨ ١٤ ٠١ ٢٨ ٦٢

LQ1594918



١١/١٠/٢٠٠٨



١٦٧٨٠١٠٣١٧٠٨٥١ ١٠/١١/٠٨



Name: جمالات عاشور مصطفى حسن		
MR. : 6154		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient attended on 30/01/2026 for initiation of a regimen comprising tamoxifen, calcitriol, and one dose of alpha-zomita 4 mg. Labs were accepted and a chest CT was requested. On 12/04/2026 the patient, a 69-year-old female, was noted to have a history of T1mic N0 breast cancer diagnosed in 2017 with subsequent PORT and adjuvant aromatase inhibitor therapy (Arimidex). In 2022 she developed a local recurrence of invasive ductal carcinoma, grade II, ER 3/8, PR 0/8, HER2 3+. She complained of abdominal pain; examination revealed left lower rib tenderness anteriorly. A CT chest/abdomen/pelvis performed on 04/04/2026 identified a hepatic segment IV focal lesion measuring 8.8 × 6.7 mm and a sacral lytic lesion. No prior comparison was available, but a CT in 12/2024 had shown a similar sacral lesion measuring 8 × 5 mm. The plan was to obtain a bone scan and provide supportive therapy. On 02/05/2026 the patient again attended for the same regimen of tamoxifen, calcitriol, and alpha-zomita 4 mg. A bone scan was pending, and the patient was advised to wait two months for pharmacy refill. On 16/05/2026 the bone scan demonstrated stable uptake at the left sacroiliac joint and several dorsal vertebrae, findings consistent with degenerative changes, and a traumatic 5th rib anteriorly. The patient was advised supportive treatment for her bony aches.		
تحريرا في ,		
	الوقت :	19-09-2026 التاريخ :
	Rana Essam الطبيب المعالج :	
يعتمد ,		



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : جمالات عاشور مصطفى ي

Sex:

3- Medical number:

Male

83060186

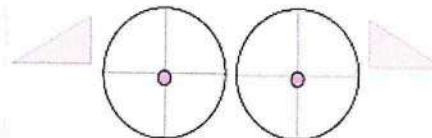
Old Med :

Old Med :

70-2021

2-ID number: 25708140102862

Patient Age : 66 Y



Family history

Family History

Phone

Phone

01100269661

BMI

BMI

REASON OF MDT

REASON OF MDT

REMDT

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

M

Date of MDT

Date Of MDT

19/12/2023

Co morbidity

Co Morbidity

Menopausal
status

Menopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :

BX
IDC

ER
3/8

PR
0/8

Her 2 Neu
3

SISH

Ki67
25

BX COMMENT:

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT :

T

N

M

1 bone

Sonomammography

PATIENT HISTORY

POSTMENOPUSAL LOCALLY RECURRENT BC TO CHEST WALL

HISTORY: RT MRM 3/5/2017,,ID TIS N0+MICRO INVASION-->ARAMIDEX

LOCAL RECURRENCE TO CHEST WALL 1/2022

PT RECEIVED 16 WK TAXOL+4 HERCEPTIN

CTS FREE FROM 6/2022-->TAMOXIFEN

PT RECEIVED 27 CYCLE HERCEPTIN

ct:SD of It SI sclerotic & lytic lesion

BS:equivocal lesion at It SI

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

continue nolvadex single agent,,, zometa

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/شريف حسنين	RADIOLOGY	RADIOLOGY د/رائيا شفيق
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



atient Name : الأستاذة / جمالات عاشور مصطفى حسن :	Request No. : 136765
atient No. : 25708140102862	Request Date : 04/01/2022
th No. : 47/2022	Verification Date : 19/01/2022
ex : Female	Financial Category : أورام دار السلام - نفقة الدولة
ge : 64 Y	Department : Out Patient
fferreng Doc. :	IHC No. :
	Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

nical Data:

operative bed mass (history of Rt MRM)

ture of the specimen:

i-Cut biopsy (of operative bed).

oss Examination:

ltiple greyish white & brown tissue cores up to 2 cm long, totally submitted.

ctions prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

roscopic examination:

istological type: Invasive duct carcinoma

rade: II (diffrentiation 3 +pleomorphism 2 + mitosis 1)

ngioinvasion: No

uctal carcinoma in situ: No

umor extent: N/A

ymph node involvement: N/A

esection Margins: N/A

dditional pathologic findings:

No microcalcification.

ignosis:

ght breast operative bed (after MRM), Tru-Cut biopsy:

vasive duct carcinoma GII.

iomarkers:

Verified by :

walaa ghoniem

Printing Date : 19/01/2022 14:48:57

Page 1 of 2

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Patient Name	الأستاذة / جمالات عاشور مصطفى حسن	Request No.	136765
Patient No.	25708140102862	Request Date	04/01/2022
Ref No.	47/2022	Verification Date	19/01/2022
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	64 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

: weak nuclear reaction in 10% of tumor cells (Allred score 3/8)
: weak nuclear reaction in 1% of tumor cells (Allred score 2/8)
r-2: strong complete membrane reaction in >10% of tumor cells (score 3+)
37: 25 % nuclear reaction

Prof.Dr. Gamal AbdelAty
Dr. Rana Fathy

Verified by :
walaa ghoniem

Printing Date : 19/01/2022 14:48:57

Page 2 of 2

Printed By : walaa ghoniem



Patient Name : الأستاذة / ديمالات عاشور مصطفى حسن	Request No. : 136765
Patient No. : 25708140102862	Request Date : 04/01/2022
Birth No. : 47/2022	Verification Date : 19/01/2022
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 64 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

operative bed mass (history of Rt MRM)

Structure of the specimen:

-Cut biopsy (of operative bed).

Gross Examination:

Multiple greyish white & brown tissue cores up to 2 cm long, totally submitted.

Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:

Histological type: Invasive duct carcinoma

Grade: II (differentiation 3 + pleomorphism 2 + mitosis 1)

Angioinvasion: No

In situ ductal carcinoma: No

Tumor extent: N/A

Lymph node involvement: N/A

Resection Margins: N/A

Additional pathologic findings:

No microcalcification.

Diagnosis:

Right breast operative bed (after MRM), Tru-Cut biopsy:

Invasive duct carcinoma GII.

Immunomarkers:

Verified by :

walaa ghoniem

Printing Date : 19/01/2022 14:48:57

Page 1 of 2

Printed By : walaa ghoniem



Patient Name الأستاذة / جمالات عاشور مصطفى حسن
Patient No. 25708140102862
Ref No. 47/2022
Sex Female
Age 64 Y
Referring Doc.

Request No. 136765
Request Date 04/01/2022
Verification Date 19/01/2022
Financial Category اورام دار السلام - نفقة الدولة
Department Out Patient
Registered by .marwa mohamed yousse

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1: weak nuclear reaction in 10% of tumor cells (Allred score 3/8)
2: weak nuclear reaction in 1% of tumor cells (Allred score 2/8)
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Prof.Dr. Gamal AbdelAty

Dr. Rana Fathy

Verified by :
walaa ghoniem

Printing Date : 19/01/2022 14:48:57

Page 2 of 2

Printed By : walaa ghoniem

Patient's Name: Jamalat Ashour Mostafa Hassan
Date: 2026-06-03
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

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OPINION:

- 1st degree degenerative anterolisthesis of L4 over L5
- Lumbar spondylotic changes with multilevel disc lesions as described.

Dr.Asser Ahmed

Dr.Asser Ahmed

Name : GAMALAT ASHOUR MOUSTAFA Date : 14-02-2026 12:55
Branch : Portable Medical No. : GRI-140226125436PM
Ref. Physician Prof : Ins. Company :

LEFT MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: *Follow up after right mastectomy.*

- **Breast density:** Scattered areas of fibroglandular tissue ... **Matching ACR b.**
- No detected suspicious mass density or architectural distortion.
- No detected pathological micro-calcification.
- **Scattered coarse and rod like calcification in the left LIQ .**
- Normal skin and nipple.

Complementary breast ultrasound revealed:

- Homogeneous fatty parenchyma of the left breast.
- **No detected suspicious solid or cystic masses.**
- No dilated ducts.
- Normal skin and nipple.
- Non specific axillary lymph nodes .
- Clear right mastectomy bed .

CONCLUSION:

- **Normal study of the left breast.BI-RADS 1 category.**

PROF.MONA ZAKY

Patient's Name: GAMALAT ASHOUR MOUSTAFA
Date: 2026-04-04
Referred by: Prof.Dr. CT CAP + C
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:

Known case of breast CA underwent right MRM. No available DICOM images for comparison.

I- CHEST:

- Status post right mastectomy with no available previous films for comparison.
- Right upper lobe subpleural reticulations likely post radiation sequelae.
- No pulmonary nodules, masses or cavitary lesions.
- Right basal atelectatic band.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.
- Bilateral elastofibroma dorsi.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. It shows a small well defined left hepatic lobe segment IV hypodense focal lesion is seen measuring 8.8 x 6.7 mm. No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Few bilateral external iliac lymph nodes largest measuring 14 x 9 mm at the right side.
- Normal appearance of the pelvic organs with no pelvic masses or collections.

Patient's Name: GAMALAT ASHOUR MOUSTAFA
Date: 2026-04-04
Referred by: Prof.Dr. CT CAP + C
Hospital: Goustave Roussey International Egypt

- No ascites.
- Tiny left ala of sacrum lytic focus.

Opinion:

- ❖ Hepatomegaly with small left hepatic lobe hypodense focal lesion for triphasic CT study.
- ❖ Tiny left sacral ala lytic focus for bone scan.

Dr.sherif Wagdy

Dr.sherif Wagdy

Name : Jamalal Athour Mustafa Hassan
Branch : Dokki
Ref.Physician Prof : Gostaf Rosch Hospital

Date : 22-10-2025 01:10
Medical No. : 231436
Ins.Company : نيل ميديكال

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 68-year-old lady, with history of right breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine
Dr. Mohamed Hassan